



Root Cause Analysis



Ruptured left tubal pregnancy with hypovolemic shock

Patient Profile: Gender Female Age 23
 Religion Buddhist Occupation Employee
 Right of treatment Cash

G2A1 GA 5⁺⁶ weeks (by LMP)

1 d PTA pelvic pain PS 3/10 with AUB
2 hr PTA while sleeping, severe pelvic pain PS 10/10 with AUB with nausea

Visit ER



Consultation



Surgery



ER

Consultation

Surgery

Complete by Timeline

☐	04.00	BP 88/52 mmHg, P 83 bpm, MAP 63 mmHg
☐	04.04	BP 80/51 mmHg, P 83 bpm, MAP 66 mmHg
☐	04.15	ER resuscitated, acetar IV load 500 mL
☐	04.18	BP 76/47 mmHg, P 80 bpm, BT 36.6 C
☐	04.19	Urine collection for UPT
☐	04.30	BP 119/56 mmHg, P 82 bpm, MAP 77 mmHg
☐	04.40	Acetar IV drip rate 120 mL

☐	04.47	Urine specimen arrived at Lab
☐	05.03	UPT positive was reported
☐	05.16	UPT result was printed
☐	05.20	BP 81/50 mmHg, P 94 bpm, MAP 59 mmHg
☐	05.20-05.22	Consult OB-GYN resident 1
☐	05.22-05.36	Resident 1 evaluated and continue resuscitate the patient
☐	05.33	(total IV: acetar 1000 mL) BP 95/52 mmHg, P 90 bpm, MAP 66 mmHg
☐	05.36-05.40	Notify resident 3 and resident 3 ordered initial management by phone
☐	05.50-06.26	Resident 3 visited at ER and evaluated and the patient
☐	05.50	BP 91/42 mmHg, P 86 bpm, MAP 58 mmHg
☐	05.52	FAST positive
☐	06.13	Chest radiography established
☐	06.26	Notify staff
☐	06.28	Set OR for EL
☐	06.30	BP drop (no record), ask for PRC 1 unit IV
☐	06.32	Nasal swab for screening COVID-19
☐	06.45	PRC was brought to ER
☐	07.01	BP 93/72 mmHg, P 86 bpm, MAP 79 mmHg
☐	07.30	(total IV: acetar 1,000mL + RLS 2,000 mL)

		BP 105/50 mmHg, P 84 bpm Patient was in OR and Anes started
☐	07.45	Anes ready
☐	07.55- 08.50	(I/O: 4,500 mL + PRC 2 units / urine 280 mL + EBL 1,500 mL) Operation time